



Allied Health Provision of Assistive Technology and Home Modifications Procedure

1. Scope

Site	Operational Area	Applicable to
Armada Kalamunda Group	Allied Health – Inpatient and Outpatient Services	Allied Health

2. Purpose

To ensure consistency in Assistive Technology (AT) provision; compliance with WA Health Policy and legislative requirements; service based on assessed clinical need; and cost-effective use of resources.

This procedure outlines eligibility requirements, items provided and the process for provision and issuing in alignment with the WA Health [Provision of Assistive Technology and Home Modifications Policy \(health.wa.gov.au\)](http://health.wa.gov.au) and relating documents.

AT and Home Modifications (HM) are provided to effect safe and timely discharge, maximise rehabilitation outcomes, assist recovery, assist unpaid carers to provide care safely, prevent readmission, reduce hospitalisation need and optimise community living viability.

AT and HM are only provided following Health Professional Assessment – within Profession Scope of Practice. Items provided must be the most basic aid/equipment which meets an essential clinical need within an eligible patient's home environment.

2.1 Inpatients and outpatients

- AT and HM provision must be related to the admission episode of care at Armada Kalamunda Group.
- Where AT/ HM need is unrelated to the reason for admission funding will be sought from alternative community or government funding. Where alternative funding is not available or not available in a timely manner and safe discharge cannot occur without provision, exceptions can be considered. These will require permission from a Head of Department.
- AT and HM will not be provided for patients awaiting surgery at other hospitals or being discharged from other hospitals. Exceptions may occur for other public hospitals if a reciprocal home assessment agreement exists or through negotiation with a Senior Therapist or Head of Department.
- Patients are considered a recent patient of Armada Kalamunda Group (AKG) and eligible for items if it is within three months since the conclusion of their last relevant episode of care and need relates to episode of care. Where patients have moved from the initial discharge location without prior planning during the admission, reassessment will not occur, and the patient will be responsible for privately sourcing any additional AT or HM required at the new location.

- Where a client has transferred to another WA Health service for ongoing treatment/therapy the new service provider is responsible for any changes in function and relevant AT provision post AKG discharge.
- Where patients are discharging to regional or interstate locations, liaison is required with the local service to where possible have the AT/HM provided in the local area. Essential AT is provided when required for safe travel. Equipment loan or extension of loans for patients moving interstate requires approval from Senior Therapist/Head of Department. Equipment approved to move with a patient interstate is issued for life of item and written off by the health service.
- Where a client discharges against medical advice (DAMA) the client has elected to discontinue service and AKG is not responsible for AT or HM provision.

2.2 Community patients

- AT and HM will be provided in line with the Community Aids and Equipment Program (CAEP) eligibility requirements, guidelines, and imprest list following funding approval from the AKG CAEP Contact.
- Armadale Kalamunda Group has no funding to provide for community patients who are ineligible for Outpatient programs or CAEP.

2.3 Department of Veteran's Affairs

- AT and HM will be provided to eligible DVA inpatients as for other inpatients. Funding approval must be obtained from DVA for items valued above \$500 prior to provision.
- Recoup of costs for DVA inpatients occurs via Department of Health/Department of Veteran Affairs agreements.
- Patients residing in the community – including outpatients who are eligible for DVA services will be directed to a DVA approved therapist for assessment and provision of any AT or HM. DVA Gold card holders are automatically DVA eligible for aids and equipment and HM whilst DVA White card holders are eligible for items related only to a DVA specified condition.
- Clients in receipt of DVA pensions who are ineligible for DVA AT and HM will be considered as per other inpatient/outpatient/community patient criteria.

2.4 Commonwealth Funding Residential Care Residents / Commonwealth My Aged Care Home Care Package Recipients and Transitional Care Programs.

- AT and HM will not be provided to clients residing in/being discharged to Commonwealth Residential Care or Commonwealth My Aged Care Home Package Recipients.
- Exceptions may apply for equipment items that constitute part of a prescribed treatment or are specialised or modified. Exceptions must be discussed with a Senior Therapist/Head of Department.
- Where an item is essential for discharge and the Commonwealth provider is unable to provide it, a short-term loan (maximum 3 months) can be provided to enable the provider to obtain the item.
- Transitional Care Program recipients cannot be provided with AT/ HM on discharge from TCP back to the community as needs now relate to the TCP episode of care.

2.5 National Disability Insurance Scheme Clients

- AT will be issued as usual for needs related to the reason for admission to ensure a safe discharge.
- When the need for an item was pre-existing to admission and unrelated to reason for admission AT will not be provided. Where an item is essential to ensure a safe discharge

and National Disability Insurance Scheme (NDIS) cannot provide emergency funding, a short-term loan (maximum 3 months) can be provided to enable the provider to obtain the item.

2.6 Social Housing Residents

- Portable aids and equipment will be issued as usual for needs related to the reason for admission, to ensure a safe discharge.
- As Community Housing Organisations (CHOs) and the Housing Authority are responsible for ensuring accessible housing for their residents, home installations and home modifications will not be provided for any AKG Outpatient clients unless it is related to an inpatient episode.
- If a home assessment for portable equipment for an AKG Outpatient, or Community client (including clients discharged from ED) is completed recommendations on installations/home modifications - including measurements can be forwarded to the relevant CHO or Housing Authority to action.
- Home assessments will not be conducted for Outpatients/ Community patients solely for the purpose of advising the Housing Authority / CHO on client installations/accessible housing requirements. The Social Housing Service will be given information on TADWA (Technology Assisting Disability WA) who will assess and then invoice the CHO or HA for their service.
- To ensure timely, safe discharge, internal home installations/ modifications will be completed by AKG for inpatients. Signed permission from the Housing Authority or CHO property manager is required prior to installations occurring.

2.7 State Funded and Non-Government Residential Care Residents

- AT can be provided for personal use by the patient but not communal use in a facility. HM are not provided.

2.8 Discharge to Rehabilitation in the Home

- Rehabilitation in the Home (RITH) is considered an extension of a hospital stay. Where equipment/HM needs change during RITH treatment, RITH therapists can **prescribe AKG items for clients whose preceding inpatient stay was at AKG**. Prescription must be in line with AKG procedures.

3. General assistive technology process

- Only AT registered on the Australian Register of Therapeutic Goods are stocked/purchased with the exception being items that do not require registration (e.g., kitchen stools).
- Items under \$50 are not provided, except where part of a prescribed treatment, specific items available at the discretion of the relevant Head of Department, or an essential accessory/part item to a prescribed item of equipment.
- Items are provided on a loan basis with the exception of items that cannot be cleaned to infection control standards e.g., foam cushions. These will be disposed of if returned.
- AT is not provided to solely meet the Occupational Safety and Health needs of paid carers – this is the responsibility of their employer.
- AT will only be issued following assessment by an appropriate AKG health professional.
- Health professionals can assess and prescribe AT/HM according to their scope of practice, areas of expertise and within service procedures and guidelines.
- Assessment must indicate an essential clinical need for the AT and it must be the most basic appropriate model to meet the patient need.

- Where an aid is useful but not essential, advice on hire/self-purchase options can be provided to the client. Equipment Purchase/Hire Hand Out can be provided to the client. Equipment Purchase/ Hire Handout
- AT can be trialled and prescribed from standard stock items.
- Where an essential need exists for a non-standard stock item this must be discussed with and approved by the relevant Senior Therapist/Head of Department to determine eligibility and necessity. Where there is sufficient clinical justification for items outside of the WA Health Policy scope or funding limits purchase approval must be obtained from the Director of Allied Health.
- The issuing health professional is responsible for, providing and documenting appropriate education and training to patient/carers in the safe use of equipment. Copies of any manuals/information provided by the manufacturer must be given to the patient. Most standard items have specific AKG care of loan equipment forms which must be provided to the patient/carer.
- The issuing health professional is responsible for setting up and adjusting equipment correctly in the home environment or providing clear instructions to the person delivering or picking up the item on how this is to be done.
- Loan length is determined by clinical need, and the prescribing health professional is responsible for discussing this with the patient/carer. Generally, loans less than 6 months are considered short-term. Loan period is indicated on the work order form for entry into PAL and the patient is provided with an Equipment Provision and Information Form. Loan equipment remains the property of AKG.
- Where a need for an item is indefinite or permanent, the AT is issued for the life of the equipment item. In these circumstances issuing therapist will provide information on repair/replacement/reassessment where patient is or will become eligible for AT maintenance via other services e.g., MAC/NDIS/CAEP.
- Where an essential clinical need is short-term, and the client is wishing to obtain an item long-term, information on the Goods Equipment and Assistive Technology (GEAT) program - for those over 65, or self-purchase information will be provided.
- The staff member issuing a stock item is responsible for ensuring it is in working order prior to issue and that all recyclable items have an intact equipment number sticker and return information sticker. Items are also checked when received into or returned to stock.
- When recyclable AT is issued the 'white homecare form' (Appendix 2), 'pink CAEP form' must be fully completed. These forms are to be given to the Occupational Therapy Assistant for recording on the Patient Appliance Loan (PAL) system for homecare items and on eCAEP for AT issued under CAEP.

4. Community Aids and Equipment Program

CAEP is a State Equipment program providing equipment to community patients with a health, disability or age-related functional impairment of a long term or permanent nature. Eligibility requirements include:

- Holding an Australian government pension concession or health care card.
- Not receiving NDIS or MAC funding or have received a compensation payment. New clients must test their eligibility for NDIS/MAC prior to accessing CAEP.
- Not a current (within 3 months) hospital inpatient or outpatient UNLESS the equipment need is unrelated to hospital episode of care.
- Reside in a home in the community or a state funded residential facility.

- Are on COSA – Continuity of Support Arrangement or DSOA – Disability Support for Older Australians.
- Residing in the AKG CAEP catchment area – please note this area differs from the AKG Hospital catchment area. [Postcode listings - Community Aids and Equipment Program \(www.wa.gov.au\)](http://www.wa.gov.au)
- Provision will be in line with the current CAEP manual and imprest list.
- All specifiers are responsible for ensuring compliance with CAEP eligibility and equipment provision requirements. Where required guidance can be sort from the AKG CAEP contact / Senior Occupational Therapy – Disability Equipment and Home Modifications.
- Specifiers can be any suitability qualified and registered internal or external health professional. The CAEP manual outlines profession requirements for specifiers for all equipment items.
- Existing clients' eligibility must be checked annually using the CAEP eligibility form prior to provision of new equipment or maintenance of existing equipment.
- All CAEP requests require approval from the AKG CAEP Contact (Senior Occupational Therapist – Disability Equipment and Home Modifications). This is to ensure eligibility checks are completed and sufficient funds are available.
- Eligibility exceptions for clients with critical equipment needs can be approved by the East Metropolitan Health Service budget holder via an out of policy request. This may include clients receiving MAC funding where there is evidence of insufficient MAC funds. Please liaise with Senior OT DEHM to determine if a client maybe an eligible exception.
- Prioritisation of funds will occur if funds are low to ensure service provision continues for high-risk needs – this may necessitate establishment of funding waitlists. Specifiers will be informed if their client is placed on a funding waitlist.

5. Handheld aids to independence (small aids)

- These can be trialled with patients however generally the patient is responsible for purchase.
- Dressing sticks can be provided to orthopaedic clients as part of their clinical pathway.
- Bradflex can be provided to build up cutlery for use by current inpatients.
- Stocking donners above \$50 in cost can be issued to eligible clients with an essential need for lower limb pressure garments.
- Exceptions may be made for items costing above \$50 if deemed basic and essential by the relevant Senior Therapist/Head of Department.

6. Mobility assistive technology

- Mobility aids issued should be prescribed to safely enable essential household ambulation.
- “4 wheeled walkers” should be issued only with approval form the Senior Therapist in consultation with the Head of Department.

6.1 Issue of transfer aid - patient lifting hoists and manual standing transfer aids

NB: All hoists and manual standing transfer aids issued require training for the patient and carer, and assessment of the home environment prior to issue. These items can only be issued by Occupational Therapy and the relevant patient/carers must be deemed competent in use of the aid prescribed.

Hoists and manual standing transfer aids will only be issued to patients who require the aid to be safely transferred between surfaces in their home environment and have a family member/carers available to and capable of safely operating the transfer aid issued.

- In exceptional circumstances hoists can be considered for patients who are falling frequently (at least weekly), whose falls cannot be reduced through other strategies and who are unable to get up from the ground independently or be assisted up from the ground safely by carers/family members without a hoist. Approval is required from Senior OT- DEHM or OT Head of Department.
- In exceptional circumstances a ceiling hoist may be considered for a CAEP eligible patient capable of independent hoist operation where there is no carer available to assist.
- Transfer aids will not be issued primarily for the use of paid carers.
- Hoists can only be issued by Occupational Therapists who have completed AKG Occupational Therapy hoist competency training.
- Hoists and manual standing transfer aids will only be issued if after training completion the carer is assessed as competent in the use of the hoist. The AKG Occupational Therapy Carer Hoist Use Assessment (MR 61.1.1 must be completed and signed by the issuing therapist and Carer.
- When carer training occurs on the ward, a home visit must have been done prior to training commencing to ensure required transfer aid can be operated in the home environment.
- Hoist slings cannot be left with a hoist in the patient's home until carer hoist competency has been attained and the hoist formally issued.
- All hoists must be issued with an appropriate, compatible sling – Two slings issued to enable washing, and drying after showering transfer.
- Standard dependent/full hoists and manual standing transfer aids are available through the home care equipment stock.
- The type of hoist and sling issued OR the type of manual standing aid, the clinical rationale for issue, and a record of training completed, must be recorded in the patients' medical record.
- Patients with an essential functional need for provision of a non-standard electrical or manual standing aid – including standing hoists must be discussed and approved by a Senior Occupational Therapist or the Head of Department Occupational Therapy prior to non-standard aid trial.

6.2 Issue of wheelchairs

NB: Wheelchair issue requires an Occupational Therapy assessment. Wheelchairs will only be issued to patients who require a wheelchair as their primary mode of mobility to enable essential function within their home.

- Wheelchairs will not be supplied either short or long term for social mobility or community access only. Options for community access include:
 - Grants may be available for these purposes via Indigo (formally Independent Living Centre).
 - Good Equipment and Assistive Technology Program (GEAT) overseen by Indigo as part of My Aged Care Funding, and
 - Patients can be offered information on private hire/purchase options.
- For palliative client or clients with fast progressing conditions, a wheelchair can be issued if the treating clinician anticipates client deterioration to requiring a wheelchair for indoor mobility within 3 months.
- Wheelchair assessment will consider clinical need, patient/carers capabilities to safely manage wheelchair, home environment, wheelchair requirements and a pressure risk assessment.

- All wheelchairs must be issued with an appropriate cushion depending on the patients assessed pressure risk i.e., minimum 50mm foam cushion with a stretch cover, and if the wheelchair is likely to be used long term (>12months) dimensions of cushion recorded on a “support surface record” to enable replacement cushions to be fabricated.
- The wheelchair issued, including a clinical rationale for issue and a record of training completed, must be recorded in the patients’ medical record.
- Wheelchairs issued on short-term loan will be sent a loan recall letter by an Occupational Therapy Assistant at the conclusion of the loan term. When a client contacts the OTA to request loan extension this will be discussed with the issuing therapist, or current outpatient therapist – where a client is under outpatient therapy care, prior to approval.
- **Standard wheelchairs** are available through standard stock. AKG relies on a limited stock of wheelchairs and immediate access to a wheelchair cannot be guaranteed. A small stock of standard wheelchair accessories is kept in the outpatient occupational therapy storeroom at Armadale and the therapy storage area at Kalamunda. These can be issued with a wheelchair when essential for a patient.
- **Non-standard wheelchairs** require initial approval for scripting/hiring from the Senior Occupational Therapist – Disability Equipment and Home Modifications. Where the need for a non-standard wheelchair is expected to be short-term hire options will be investigated to determine if this is more cost effective than purchase. Where required AKG inpatients and outpatients can be referred to the Senior OT DEHM for specialist scripting of long-term loan wheelchairs. Purchase of non-standard wheelchairs for AKG inpatients and outpatients requires approval from the Head of Department Occupational Therapy.
- **Power wheelchairs** in stock are primarily CAEP owned. These can be used for trial purposes following discussion with the Senior Occupational Therapist – Disability Equipment and Home Modifications. Power wheelchair issue/purchase for hospital patients requires approval from Head of Department Occupational Therapy.
- **Paediatric wheelchairs:** A small stock of adjustable wheelchairs is available for issue to children for short term loan only - usually 6 weeks. These are for children with lower limb fractures/injuries who are unable to use crutches and are too old for a pram and their parents are unable to carry them. Elevating leg rests are kept in stock for these wheelchairs.

6.3 Issue of seated support surfaces

NB: Issue of support surfaces requires an Occupational Therapy assessment and completion of a multidisciplinary Comprehensive Pressure Injury Risk Assessment/Screening.

- Refer to Occupational Therapy Pressure Injury Prevention and Management Procedure [Occupational Therapy Pressure Injury Prevention and Management](#) for instructions on pressure injury risk assessment.
- Basic foam cushions can be issued and/or manufactured for non-pressure care needs as clinically indicated including positioning needs, functional transfers, specified orthopaedic precautions or pain management e.g., fractured coccyx.
- All potential long-term cushion loans and all high-risk cushion loans require completion of a “support surface record” to enable replication of the cushion in the future.

6.4 Issue of high-risk pressure support surfaces – bed and seating

- **Criteria for use**
 - Current Stage 3 or above pressure injury.
 - Braden Scale Score of 14 or below.

- Previous pressure injuries OR current pressure injury AND at continued high risk AND a lower risk pressure surface is unsuitable.

OR

- A clinical reason that has been discussed with the Senior Occupational Therapist – Disability Equipment and Home Modifications, such as prolapse, or postural issues.
- All short-term loan cushions issued for non-palliative clients from Armadale will receive telephone assessment via post discharge clinic within 3 months of the issue of the cushion. A referral for the client needs to be made at time of issue of the cushion to enable this to occur.
- Kalamunda does not have a stock of high-risk cushions for home issue. Where this is essential, OT can arrange from Armadale stock.
- To issue a high-risk pressure support surface for a term longer than 3 months the patient must be at high risk of having a pressure injury AND be expected to remain high risk in the future. Approval is required from a Senior Occupational Therapist or Head of Department.
- High risk cushions issued for longer and 3 months must be pressure mapped and determined as the most suitable for the patient at the current time.
- Kalamunda does not have a pressure mapper. For process to loan refer to [Occupational Therapy Pressure Injury Prevention and Management](#) procedure
- **Beds:** Are not provided by AKG but grants may be available. A Bed Provision information handout can be provided to patients wishing to self-purchase/hire.
- **Bed raisers:** Fabricated raisers are only issued for permanent long-term need. Portable plastic raisers will be used for all short-term users unless deemed unsafe.
- **Portable bed rails:** All portable bed rails require a risk assessment by an Occupational Therapist in the home prior to issue. Guidance for clinical judgement including direction on when bed rails are not to be issued or issued with caution is provided in the AKG [Occupational Therapy Issue of Bed Rails for Home Use Guidelines](#)

6.5 Issue of seating

- One chair set-up per client – utility chair OR high back chair.
- One kitchen stool can be issued where there is an essential need for the person to complete kitchen duties.
- Chairs require a clinical justification and cannot be issued where chair is only required as a normal household item.
- High back chairs are generally only issued for a maximum of 3 months and can be issued as an interim whilst chair raise is fabricated or while patient purchases a more suitable chair.
- Long term issue of high back chairs requires a clinical justification for issue related to an ongoing health/disability condition. Where requirement is for additional chair height only a chair raise should be considered.
 - Patient's over 65 may be eligible for assistance for a suitable chair via GEAT.
 - Client owned suitable single chairs can have raise built to fit. Raises for couches will only be considered where there is no other option and require approval from Senior Occupational Therapist – Disability Equipment and Home Modifications.

6.6 Repair and replacement of AT

- Repairs of AT is only undertaken to maintain the safety and function of the AT, not for cosmetic reasons.

- Repairs and replacement will only occur for reasonable wear and tear, general maintenance requirements, accidental damage as part of normal use or where a manufacturing fault is identified with the AT.
- The decision to either repair or replace an item is based on the most cost-effective option considering logistics and timeliness of repair/replacement.
- Replacement requests related to a change in patients' needs rather than maintenance require reassessment by an appropriate health professional. Patients who are no longer a current AKG inpatient/outpatient/CAEP patient will be given information on where and how to obtain a referral for reassessment.
- Repairs and replacement will not be considered where AT has been used outside issuing instructions or where damage was deliberate. Exceptions will be considered in circumstances which were outside the users control and should be discussed with relevant Head of Department.

Criteria for repairs:

- AKG will organise repair or replacement in lieu of repairs for AT on a non-expired loan where:
 - AT issued on a short-term loan for a time limited condition ie under 6 months
 - AT issued within the last 3 months with any loan length.
- Where a loan has expired:
 - If it is deemed that a patient no longer requires an item as per AKG issuing criteria the item will require return to AKG.
 - Repairs will be considered where the Senior OT DEHM or relevant treating therapist determines the loan requires extension.
- For AT issued on long-term/life of equipment loans:
 - Redirection to other services or programs where eligible, including MAC, NDIS, CAEP.
 - Where client is ineligible for any other services repairs will be organised by AKG in situations where AT is still required by the patient and not repairing/replacing the AT may lead to injury, medical emergency, or hospital admission.
 - Repairs and replacement cannot be undertaken for patients who have moved to regional areas or interstate.

6.7 Return of AT and Disposal

- All AT issued with an equipment sticker remains AKG property and requires return when no longer required.
- Where able patients/ patients' families are encouraged to return AT to AKG.
- AKG AT for patients in regional or interstate areas if unable to be returned by the patient to AKG is taken off PAL / eCAEP systems and patient is advised that item can be donated to their local health service if in good working order and no longer required.
- AKG OT department can pick-up equipment for those unable to return AT within the metropolitan area. Phone number to organise return is on the equipment sticker.
 - Waitlists exist for pick-up and patients are informed of approximate wait times.
 - Pick-ups for deceased patients are prioritised.
 - Couriers can be utilised where more economical due to distance or for timing/logistical reasons as determined by Senior OT – DEHM.

- Equipment returned is inspected for safety and function.
 - AT suitable for reissue are cleaned and returned to stock.
 - AT able to be economically repaired are repaired and returned to stock.
- AT deemed unserviceable on return are removed from PAL / eCAEP.
 - Salvageable parts are removed and stored in OT shed.
 - RPH RTU can be contacted, and photos sent to determine if any wheelchair parts are required by their service.
 - Items to be disposed of:
 - All electrical items need the electrical cord cut off prior to disposal.
 - Large items go to the skip at the rear of the hospital outside the fleet care compound. The Home Care Carpenter can assist with disposal into the skip. Remember to use good manual handling techniques and ramp on skip bin.

7. Home installation / modifications

- All home installations/modifications require Occupational Therapy assessment of the home environment and patient.
- [Occupational Therapy Home Installations, Fabrications and Modifications Guidelines](#) provide additional clinical guidance information and requirements for homeowner permission
- The white home care form, or the pink CAEP home care request form are completed to action all installations/modifications. Additional pages/photographs can be attached to this form. For guidance on photograph requirements refer to AKG [Occupational Therapy Home Installations, Fabrications and Modifications Guidelines](#)
 - All Armadale health service home care request forms are checked by the Senior Occupational Therapist – Disability Equipment and Home Modifications prior to allocation to the home care carpenter or external contractor.
 - The Home Care Carpenter/External Contractor will contact the prescribing Occupational Therapist if an installation/modification is unable to be completed or need to be altered from the original prescription
- Installations versus portable equipment use will be on a clinical and cost benefit basis.
- Generally basic portable equipment is provided in preference to home installations/modifications – particularly where portable equipment issue will negate the need for an Occupational Therapy home assessment.
- Repair/replacement will occur for any installation or structural modification undertaken by the Home Care Carpenter for up to six months following the completion of the work. After this time, it becomes the client's responsibility to undertake maintenance.
- Repairs for work undertaken by contractors in the previous 6 months is directed to the contractor to rectify.
- For hospital inpatients and hospital outpatients any single installation/modification over \$1500 a 10% contribution from the client will be requested.
 - **Home installations which will disturb asbestos (drilling) are not provided by AKG due to health risks to the client and installer.**
 - Where major structural modifications to an asbestos home have been approved a registered builder with an **Asbestos Removal Licence** must be used.

7.1 Structural home modifications

- Clinical criteria is provided in the AKG Occupational Therapy Home Installations, Fabrications and Modifications Guidelines. A registered builder is required to complete structural modifications.
- Hospital patients
 - Minimum 10% contribution is required from the client if over \$1,500 for a single modification.
 - Maximum AKG funding limit is \$6000.
 - Must be assessed with the Senior Occupational Therapist – Disability Equipment and Home Modifications. Final approval will be required from AKG Director of Allied Health.

7.2 Priority system for equipment delivery and home installations / modifications

- Homecare request forms require allocation of a completion priority 1-3. Where installation completion is essential for discharge anticipated discharge date must be written next to the priority.
- At Armadale notification of completed home installation requests is completed by the OTA/Carpenter via email.
- At Kalamunda OT receives confirmation of completion direct from the installers.

7.2.1 Priority 1: Within 7 working days.

- Patients or their carers/family members being at risk of hospitalisation, serious injury or entering hostel or nursing home care without completion of the work. i.e. patients:
 - Who are frequently falling
 - Who are living alone and at risk due to no social support.
 - Who are medically unstable/have fluctuating abilities e.g. terminal diagnosis.
 - Require alterations to be completed pre-discharge.
 - Note large fabrications may require longer than 10 days to complete.

Installations required in less than 7 days cannot be guaranteed – these should be requested on the home care form and discussed with Senior Occupational Therapist – Disability Equipment and Home Modifications.

7.2.2 Priority 2: Within 20 working days.

- Patients who can complete tasks either with or without support but will be more independent or be able to complete tasks with less difficulty when work is complete. i.e., patients:
 - With deteriorating medical conditions (i.e., MS, RA, MND) residing within a supportive environment
 - Requiring assistance from Care Organisations and placement of rails etc. will assist their safety and care.
 - Carer under stress which can be relieved through appropriate environmental change.
 - Who have fallen, however falling risk is not frequent.
- Being discharged from a ward however can wait for work to be completed.

7.2.3 Priority 3: Within 30 working days.

- Preventative interventions.

- Pick-up of equipment – most of this will be carried out by the OTA once a week. Equipment pick up is only available when clients/family members cannot return equipment independently.

8. Roles and responsibilities

Role	Responsibility
Orientation to Assistive Technology	• All OT staff to undertake on-line education or attend presentations by Senior Staff for new starters, for general & complex AT & HM including: Hoist, Wheelchairs, Bed Rails, Pressure relieving Devices, Home Assessments, Home Modifications. • All OT Staff to complete the on-line competencies and practical hoist competency, with completion certificate sent to HOD OT for competency record log. • All ED OT and PT staff to complete competencies for generic equipment/mobility device prescription at orientation to ED.
Senior OTs DEHM	• Provide education on complex equipment, • Maintain online education/competencies for currency, and • Provide education and advice for OT/PT staff as required on the implementation of this policy & complex, out of scope requirements.
All AT prescribers	• To complete an accurate Work Order form whenever equipment is issued including the equipment number and client's signature to enable accurate PAL records.

5. Legislative context

The following documents are required to give effect to this procedure:

- Occupational Safety & Health Act 1984
- [WA Health Pressure Injury Prevention & Management Clinical Guideline](#)
- [Arrangements for the Provision and Charging for the Department of Veterans' Affairs \(DVA\) Entitled Persons](#)

9. Supporting information

The following documents support the implementation of this procedure:

- [AKG Pressure Injury Prevention and Management](#)
- [Occupational Therapy Pressure Injury Prevention and Management](#)
- [Occupational Therapy Issue of Bed Rails for Home Use](#)
- [Occupational Therapy: Home Installations, Fabrications and Modifications](#)

10. Compliance, monitoring and evaluation

All AKG Occupational Therapy staff are required to complete basic AKG competency training in the issue of Occupational Therapy aids and equipment, including specific training for

wheelchairs, bedrails, hoists, home assessment for installations/modifications and pressure care support surfaces.

10.1 Monitoring

Compliance against this policy will be evaluated in conjunction with relevant departmental medical record clinical content audits.

Compliance for home installations/modifications is ongoing via the work order request authorisation process by the Senior Occupational Therapist – Disability Equipment and Home Modifications.

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard when providing comprehensive care to:
 - a. Actively involve patients in their own care
 - b. Meet the patient's information needs
 - c. Share decision-making
- Standard 5 – Action 5.13 Developing the comprehensive care plan
- Clinicians use processes for shared decision making to develop and document a comprehensive and individualised plan that:
 - a. Addresses the significance and complexity of the patient's health issues and risks of harm
 - b. Identifies agreed goals and actions for the patient's treatment and care
 - c. Identifies the support people a patient wants involved in communications and decision-making about their care
 - d. Commences discharge planning at the beginning of the episode of care
 - e. Includes a plan for referral to follow-up services, if appropriate and available
 - f. Is consistent with best practice and evidence
- Standard 6 – Action 6.11 Documentation of information
- The health service organisation has processes to contemporaneously document information in the healthcare record, including:
 - a. Critical information, alerts and risks
 - b. Reassessment processes and outcomes
 - c. Changes to the care plan

12. References

1. Department of Social Services. (2014). *Care and services in aged care homes*. Retrieved from Australian Government Department of Health: Ageing and Aged Care: https://agedcare.govcms.gov.au/sites/g/files/net1426/f/documents/09_2014/additional_guidance_-_care_and_services_in_aged_care_homes_0.pdf
2. Fiona Stanley Hospital. (2016, January). Provision of Equipment, Home Modifications and Installations.
3. Minister for Social Services. (2014, June). *Quality of care principles 2014*. Retrieved from Federal Register of Legislation: <https://www.legislation.gov.au/Details/F2014L00830>

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13. Glossary

The following definitions are relevant to this policy.

Term	Definition
Assistive Technology	Portable, movable, or freestanding items that assist in function or safety in activities of daily living.
Patient Lifting Hoist	An assistive device that allows patients to be transferred between bed and chair or other similar resting places using hydraulic power. They can be mobile, or ceiling mounted and lifts in a standing, sitting, or lying position.
Wheelchair	Wheeled device to transport clients in a seated position from place to place when they are unable to walk that distance. They can be manual (transit, self-propel, and/or power assisted) or power operated (with or without attendant control).
Installations	Items requiring fixing to a wall, floor, or other surface.
Structural Modifications	Structural changes to layout or fixtures in the home.
Home Modifications	Term encompassing installations and structural modification.

14. Policy contact

Head of Department Occupational Therapy

Enquiries relating to this procedure may be directed to:

Title: Senior Occupational Therapist Disability Equipment and Home Modifications
 Department: Occupational Therapy

15. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V3	01/09/2023	01/09/2026	Scheduled review

16. Authorisation

This procedure has been authorised and issued by the Director Allied Health.

Authorised by	My Linh Nguyen – A/Director Allied Health
Authorisation date	31/08/2023
Published date	01/09/2023

Appendix 1: Equipment Provision and Information for Client's 65+

You have been issued with the following equipment on the _____

- _____
- _____
- _____
- _____
- _____

7. Equipment provided by Armadale Kalamunda Group is **issued for the life of the equipment** however we are **only able to repair equipment within 6 months of issue**. Please contact Occupational Therapy if you need equipment repair within the first 6 months of issue by telephoning 9391 2393.
8. If your equipment has been issued for over 6 months and it requires replacement you will need to register with My Aged Care to access the **Goods, Equipment and Assistive Technology Program** known as **GEAT**, or maintain the equipment yourself. See below contact information for My Aged Care.
9. If you commence a My Aged Care Package of care, please contact your package provider for all repairs and future equipment needs.
10. If you are not receiving a My Aged Care Package, you will need to **register** for the Community Home Support Program (CHSP) for **repairs, maintenance, replacement of your equipment** and future equipment needs.

Register directly at the **My Aged Care Website**. This service will arrange the assessment and referral to the appropriate agency for home installations and/or equipment.

- Referral form and information is available via:
- <https://www.myagedcare.gov.au/assessment/apply-online>
- Phone: 1800 200 422

Occupational Therapy
Armadale Kalamunda Group
3056 Albany Highway ARMADALE Western Australia 6112
PO Box 460 ARMADALE Western Australia 6992
Telephone (08) 9391 2392 Facsimile (08) 9391 2262

Appendix 2: Home care

PRIORITY: 1 2 3

**ARMADALE KALAMUNDA GROUP
HOME CARE EQUIPMENT PROGRAM REQUEST FORM**

DATE: _____

TYPE OF SERVICE: A. EQUIPMENT [] SUPPLY [] RETURN
 B. MAINTANENCE [] CONSTRUCT [] INSTALL
 (Details over page)

PATIENT DETAILS:

Title: Mr/Ms/Miss/Mrs MR NO: _____

Surname: _____

First Name: _____

D.O.B: _____

Address: _____

Phone: _____

LOAN DETAILS:

Clinician name: _____

Dept/Unit: _____

☐ OT ☐ PT
 CR [] CSRU I/PT [] COLYER []
 CWL [] MH [] MAU [] CANNING [] ED []
 ICU [] RITH [] KAL []

Loan Term (recyclable equip. only):

☐ Short Term (no of wks until return) _____☐ Palliative☐ Long Term**NEXT OF KIN:** (Not living with the client)

Name: _____

Relationship: _____

Address: _____

EQUIPMENT SUPPLY/RETURN

Supply/loan: Please deliver the following equipment to patient [] to authorising officer []

Equipment has already been supplied by authorising officer []

Return: Please collect and return to cleaning shed [] Returned to OT shed via authorising officer []

SUPPLY	RETURN	QTY	DESCRIPTION	H.C. NO.	COST

☐ **Client is on an existing/going home with a Commonwealth Package of Care**

Comments: _____

Items supplied/delivered by: (Signature) _____

Signature of person receiving goods: _____

Priority: **1 2 3** Expected Discharge Date: _____ Therapist: _____

Patient Details

Title: Mr/Ms/Miss/Mrs

UMRN: _____ DOB: _____

Surname: _____ First Name: _____

Address: _____

Home No: _____ Mob No: _____

Best Contact: _____

CONSTRUCTION/INSTALLATIONS/MODIFICATIONS/MAINTANENCE REQUIRED

Type of House: ☐ Brick ☐ Brick Veneer ☐ Timber ☐ Timber/asbestos ☐ Other _____

Permission from Homeswest/Landlord: _____

I give my consent for Armadale Health Service to:

- Take photographs where home modifications are required and
- Complete the home installations/modifications detailed below:

Home owner signature: _____

Information required for shower hose installation

Electrical fixtures – Distance from shower rose	mm	mm
Light switches		
Power points		
Lights - Covered Y / N		
Heaters		
Is there an RCD installed?		

Comments: _____

W:\Aged Care\AKH\AGE\OT\Equipment\Forms\WorkOrders\HomeCareloanform.doc